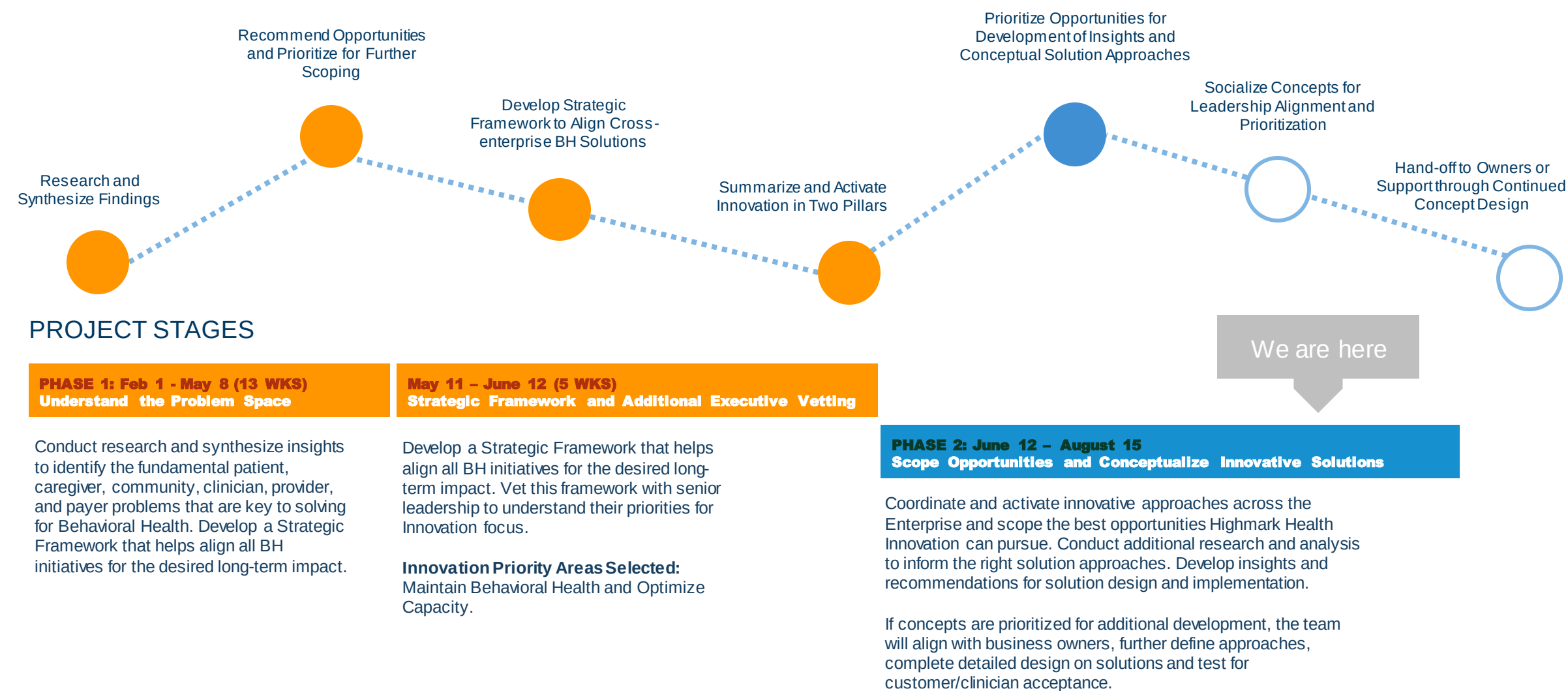


Behavioral Health Innovation Process



Phase 2 Innovation Process

To define Innovation Opportunities and Solution Concepts in this phase, the Innovation group engaged in...



30+ Clinician Interviews

BH Clinicians, PCPs and Specialists across AHN and with our Partners



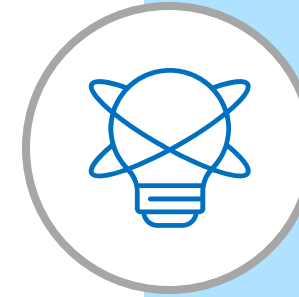
6 External Expert Conversations

Forrester Analysts, Startups, University and Clinical Partners, and National Advocacy Groups



30+ Customer Interviews and Concept Tests

Research included customers of Highmark, AHN, and local/national competitors



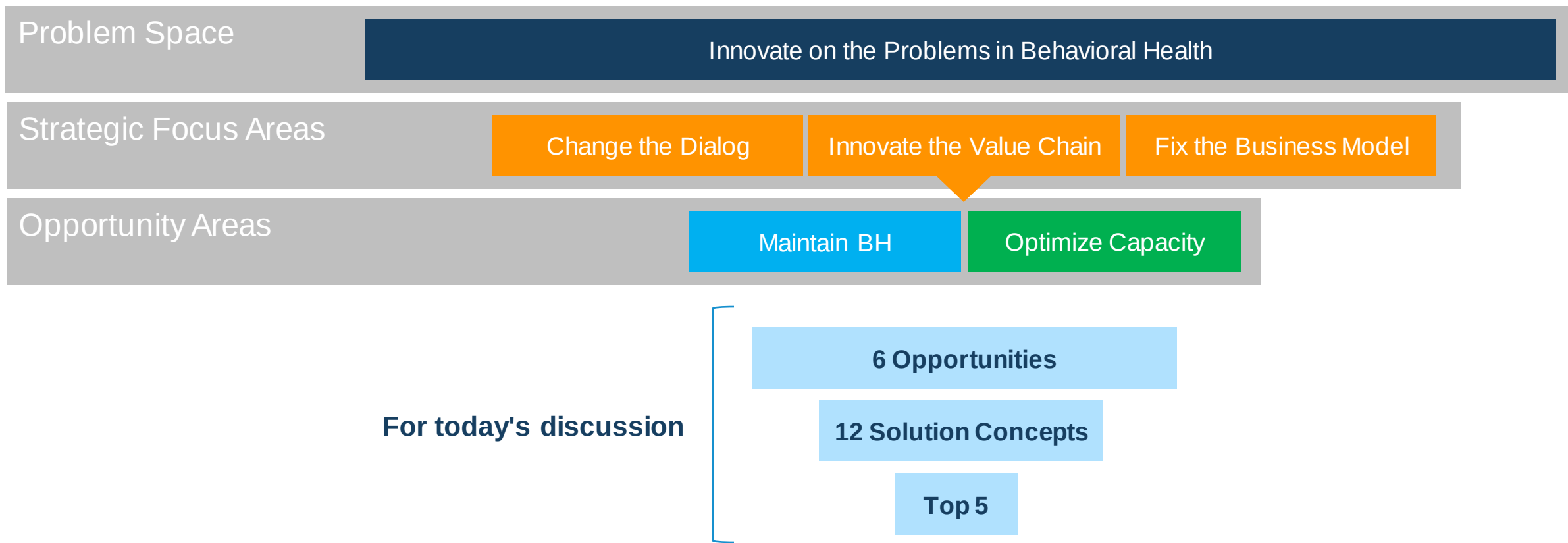
6 Ideation Sessions w/ 80+ Cross-Enterprise and External Participants

Including a session with 50+ BCBSA participants from 14 plans

Prioritized Innovation Opportunities

We started with a huge Problem Space and have been continuously narrowing our focus through learning and prioritization.

Broad Focus in Phase 1



Six Prioritized Innovation Opportunities

Maintain Behavioral Health

Opportunity 1

Prevent and intervene early through awareness and self-care

Opportunity 2

Bring new approaches to identifying BH needs and activating care

Opportunity 3

Activate and resource the entire caregiver community to achieve greater scale

Opportunity 4

Align the continuum of needs with the right levels of care

Optimize Capacity

Opportunity 5

Be a leader in BH clinical insights through research

Opportunity 6

Address the needs of underserved populations and provide equitable access to culturally competent care

12 Solution Concepts and Prioritization

Twelve ideas, generated from collaborative sessions, were developed into concepts to solve for BH opportunities

Opp 1 - Concept 1 Personalized BH Box Create a personalized “box” that provides tailored knowledge and resources.	Score 6.87	Opp 1 - Concept 2 Active BH Learning Educate people on mental wellness through active engagement channels.	Score 7.62	Opp 1 - Concept 3 Peer Support—Anywhere Offline and online platform to connect people with peers for support and community building.	Score 9.49	Opp 2 - Concept 1 Identify Through Data Leverage wearable, claims, and EMR data in novel ways for early outreach and intervention.	Score 8.18
Opp 2 - Concept 2 Perpetual Screening Ubiquitous, simplified screening across the clinical footprint.	Score 9.09	Opp 2 - Concept 3 BH Care Hub Streamline care activation through transparent, simple information and tools.	Score 8.73	Opp 3 - Concept 1 Multichannel BH media Bringing BH education, resources, and tools in a low-barrier format for everyone.	Score 7.07	Opp 3 - Concept 2 Gamified BH A multiplayer role-playing game with BH elements to build BH resiliency for youth (and others).	Score 7.98
Opp 4 - Concept 1 Navigational Pathways Platform to help patient & provider understand existing resources, types of therapies, and next steps in care.	Score 8.89	Opp 5 - Concept 1 Dedicated BH Research A dedicated BH data analytics work group made up a multi-disciplinary team that continually rotates over time.	Score 8.53	Opp 6 - Concept 2 Provide a Support System Ease individuals into trusting the system and seeking care to support them along their journey.	Score 8.38	Opp 6 - Concept 2 Culturally Competent Access Create true access to all those who currently lack it. Expand our pipeline of diverse clinicians and clinical competency as an organization.	Score 8.73

Prioritization Scoring: 3 Components / 5 Measures

Each concept was scored on five measures and validated with our clinical team. Scores were combined and normalized on a 1-10 scale. See appendix for details.

IMPACT =

Reach: The number of people we can touch with the solution

Outcome: The scope of the outcome the solution is expected to achieve

DIFFICULTY =

Time/Effort: The time/effort needed to build and launch the solution

Cost: The cost required to launch the solution

SCALABILITY =

The ability of the solution to scale beyond AHN

Top-Five Scoring Solution Concepts

Five concepts scored highest on components of Impact, Difficulty and Scalability.

1

BH Care Hub
Streamline care activation through transparent, simple information and tools

Score 8.73

2

Perpetual Screening
Ubiquitous, simplified self-screening across the clinical footprint

Score 9.09

3

Peer Support—Anywhere
Offline and online platform to connect people with peers for support.

Score 9.49

4

Navigational Pathways
Platform to help patient & provider understand existing resources, types of therapies, and next steps in care

Score 8.89

5

Culturally Competent Care
Create true access to all those who currently lack it. Expand our pipeline of diverse clinicians and clinical competency as an organization.

Score 8.73

Foundational BH Solution Concept

The cornerstone of our solution concepts is the BH Hub—creating an integration, access, and launching point for experiences proposed in other concepts and for all other BH solutions selected in the Living Health.

The Hub becomes a consolidation and curation point for Behavioral Health Solutions—potentially developed on the Living Health Platform and integrating seamlessly into the overall Living Health Customer and Clinician Experience. **We don't envision this as a standalone platform.**



*Highmark App is not the Hub...
Highmark.com is not the Hub...
MyStrength is not the Hub...
The EMR is not the Hub...
Virtual Care is not the Hub...*

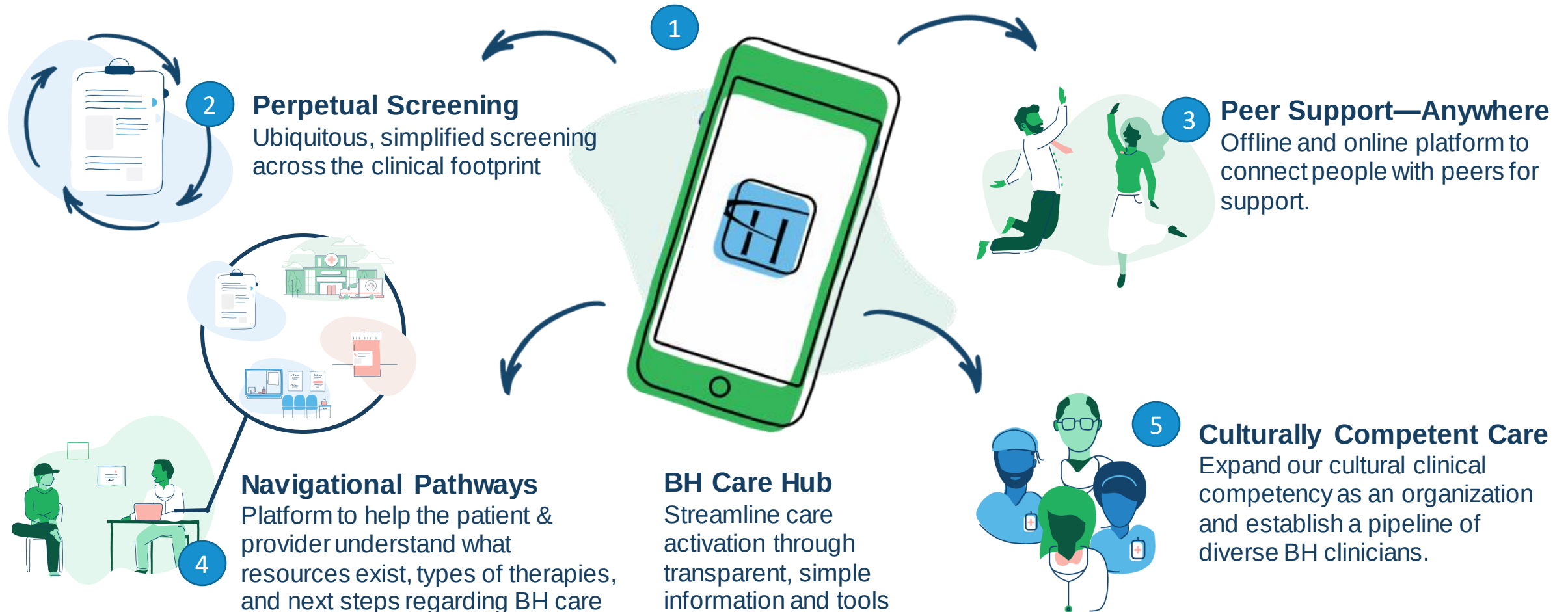
But the BH Hub can connect and integrate with these digital tools and experiences to create the simple, single access-point for people who seek BH care and resources.

BH Care Hub

Streamline care activation through transparent, simple information and tools

Top-Five Concepts as a BH Ecosystem with Broad Impact

The additional four concepts can be integrated with and delivered to customers and clinicians via the BH Care Hub for an ecosystem solution that has impact across five BH Opportunity areas.



Solution Concepts Address Five of the Six Opportunities

Maintain Behavioral Health

Opportunity 1

Prevent and intervene early through awareness and self-care

Opportunity 2

Bring new approaches to identifying BH needs and activating care

Opportunity 3

Activate and resource the entire caregiver community to achieve greater scale

Opportunity 4

Align the continuum of needs with the right levels of care

Optimize Capacity

Opportunity 5

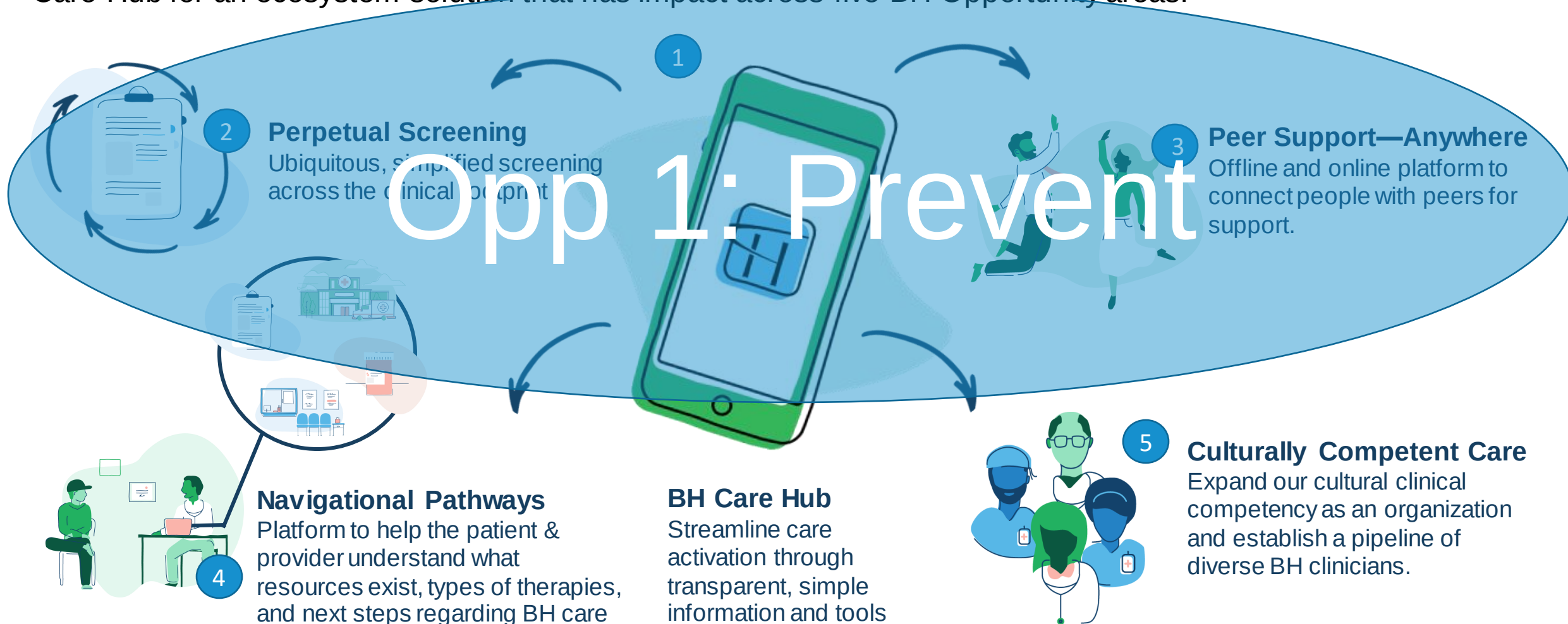
Be a leader in BH clinical insights through research

Opportunity 6

Address the needs of underserved populations and provide equitable access to culturally competent care

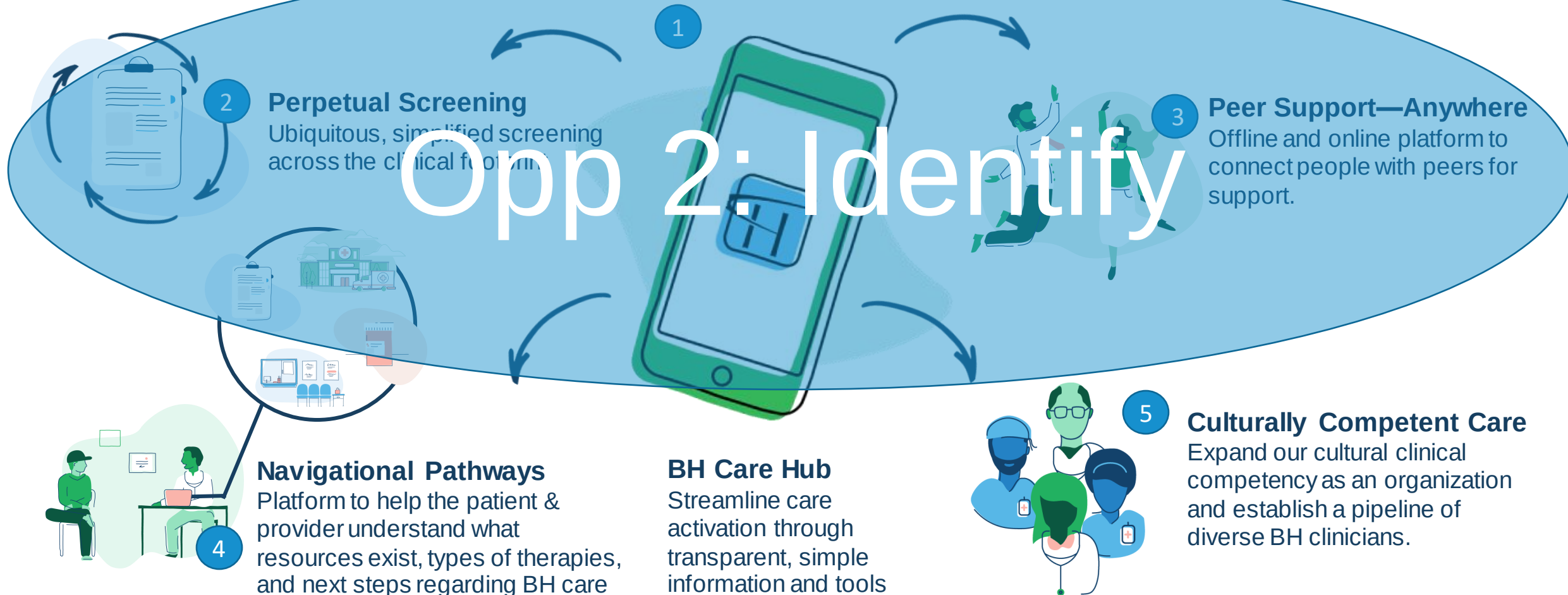
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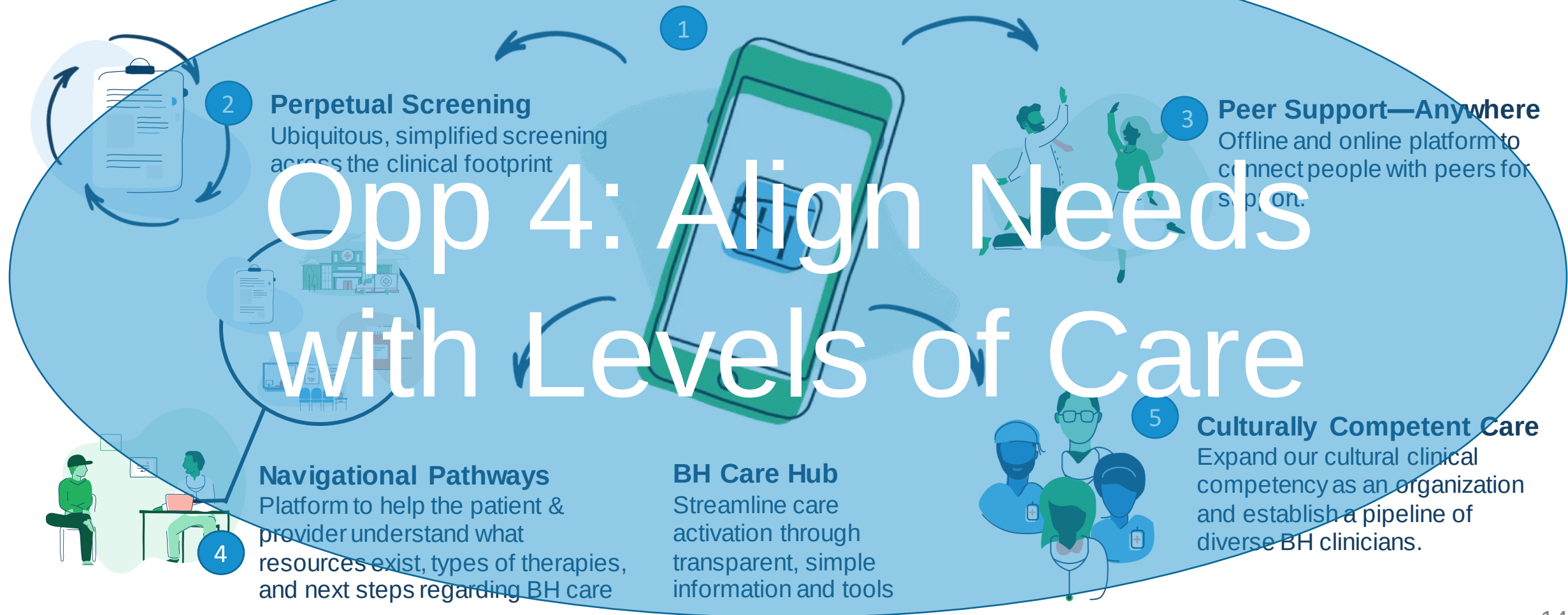
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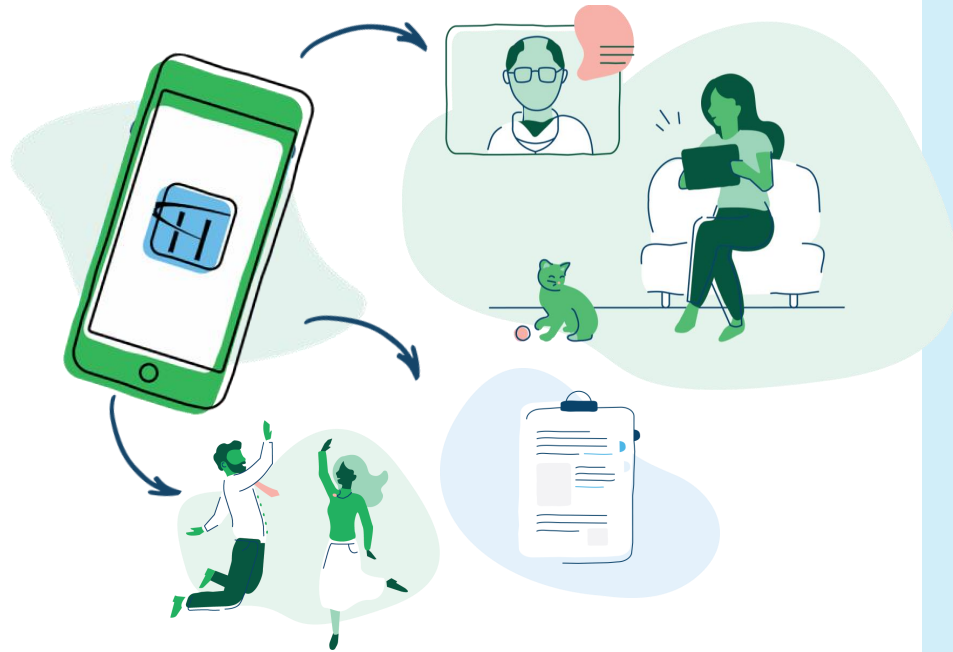
The additional four concepts can be integrated with and delivered to customers and clinicians via the BH Care Hub for an ecosystem solution that has impact across five BH Opportunity areas.



BH Care Hub

Concepts summarized at-a-glance

Streamline care activation through transparent, simple information and tools



A digital resource hub that makes accessing and maintaining BH care and resources simple, straightforward, and non-intimidating.

Unmet Needs It Satisfies

Allows people seeking help to easily activate BH care in a safe, non-threatening space that gives them the information they need to make informed decisions about their care.

Key Features

- Up-to-date, robust, provider (credential and specialty) information and scheduling options with in-network BH providers
- Curated BH wellness information, resources, and tools, including peer groups and community services
- Information on actionable next steps and care plans
- Available BH providers for real-time telehealth (through access to existing virtual care platform)

Value Provided

- Serves both customers and clinicians
- Establishes an easily navigable and accessible platform for all BH care and resources
- Curates and consolidates point-of-access for existing and to-be-developed BH solutions
- Increases brand loyalty
- Increases engagement and satisfaction
- Nationally scalable, but requires some localized content about providers and resources
- Provides commercialization opportunity—first to provider partners and other blues

Ballpark Investment

\$\$ - Small investment to MVP and pilot, medium investment to scale

Perpetual BH Screening

Concepts summarized at-a-glance

Ubiquitous, simplified self-screening across the clinical footprint



Simplified screening tools leveraged across our entire clinical footprint that allow non-BH clinicians to easily recognize and address BH illness onset and provide opportunities for patients to self-screen in the privacy of their own home.

Unmet Needs It Satisfies

Helps non-BH clinicians and their patients recognize when symptoms need to be addressed. Takes the time and expertise gap burdens off of clinicians doing BH screenings and empowers patients to self-activate care on their own time.

Key Features

- Simplified, staged, qualitative questionnaire that can be completed digitally at home or prior to appointments
- Provides real-time info for patients and providers on available next steps, based on symptoms
- Available to all non-BH clinicians and practices
- Delivered through low-barrier channels (SMS, BH Hub)

Value Provided

- Serves customers and clinicians
- Becomes the scalable structure for Highmark/AHN's early intervention efforts
- Reduces costs of care over time through early intervention
- Increases engagement and satisfaction
- Nationally scalable
- Provides commercialization opportunity—first to provider partners and other blues

Ballpark Investment

\$\$ - Small investment to MVP and pilot, medium investment to scale

Peer Support—Anywhere

Concepts summarized at-a-glance

Offline and online platform to connect people with peers for support



Connect individuals with peers offline and/or online to better identify, prevent and self-manage the onset or escalations of BH conditions

Unmet Needs It Satisfies

Helping members and patients connect with other people who share similar backgrounds, experiences, and/or conditions for support.

Key Features

- Platform to help facilitate communications and interactions
- Private channels for communication with peers
- Potential to provide coordinated peer-counselor experiences through AHN
- Peer-counselors become champions and advocates that can help connect customers with knowledge, resources and care providers
- Peer support is a low-cost and highly effective mode of care

Value Provided

- Serves customers and expands capacity for care
- Becomes the structure for scaling a peer-support program beyond BH
- Reduces costs of care over time through early intervention and low-cost care through peer support
- Increases engagement and satisfaction
- Nationally scalable with peer-support partners (Axial, MAP Healthcare, Steel Smiling)
- Provides commercialization opportunity—first to provider partners and other blues

Ballpark Investment

\$\$\$ - Medium investment to MVP and pilot, large investment to scale and introduce more structured peer-counselor programming

Navigational Pathways

Concepts summarized at-a-glance

Platform to help patient & provider understand existing resources, types of therapies, and next steps in care



A cross-system digital “Sherpa” that educates and informs along the care continuum. It enables greater engagement through care transitions and delivers low-barrier, actionable, and clear next steps to patients, caregivers, and providers.

Unmet Needs It Satisfies

Helps customers increase engagement in the disease state, standardizes treatment, and reduces stigma and barriers by making BH care pathways clear and simple.

Key Features

- BH Care Pathway Management
- BH-Specific Discharge Plans
- BH Condition and Continuum Management Tools for clinician and customer use

Value Provided

- Serves customers and clinicians
- Empowers non-BH clinicians to more confidently propose care options
- Reduces costs of care over time through early intervention and more standardized/evidence-based treatments
- Measures and incentivizes appropriate adoption of evidence-based treatments
- Provides commercialization opportunity—first to provider partners

Ballpark Investment

\$\$\$ - Medium investment to MVP and pilot, large investment to scale and commercialization with a reliance on the data analytics foundational capabilities with ongoing analytics-driven care pathway improvements.

Culturally Competent Care

Concepts summarized at-a-glance

Enhance the diversity of our clinical BH network and elevate cultural competency of BH Clinicians



It's necessary for providers to deliver Culturally Competent Care in order to form strong patient relationships and meet the BH needs of African-American, and other underserved and under-represented, communities and individuals.

Unmet Needs It Satisfies

Low efficacy of BH care for African-American populations; shortage of diverse providers; low level of cultural competency in existing provider network

Key Features

- Educational content to increase empathy and practical skills allowing providers to deliver more effective BH care.
- Trainings and certification to expand provider skillsets to include therapies specific to race-based patient experiences.
- Longitudinal plan for AHN BH workforce diversification involving community engagement and redesigned clinical pipeline, recruiting, and hiring practices.

Value Provided

- Can expand and improve care of other underserved markets and raise the level of care for everyone
- Improves relationship between BH system and African American communities
- Impacts our existing provider shortage
- Reduces costs of care over time through early intervention and more standardized/evidence-based treatments
- Provides commercialization opportunity—first to provider partners

Ballpark Investment

\$ - Small investment to build and rollout clinician training, with potential for medium investment for a broader AHN clinical certification and longitudinal workforce diversification plan

Preliminary BH Ecosystem Roadmap

Proposed Staging of Design and Development Activities				
Concept	Cost	0-6 m.	6-12 m.	12-24 m.
BH Care Hub Streamline care activation through transparent, simple information and tools	\$\$	- Internal collaboration to create detailed design and prototype of hub pilot/MVP - Integrate existing BH resources/tools - Plan/launch effort to collect detailed specialty info for in-network BH clinicians in WPA	- Create marketing plan for customers and clinicians - Develop and launch hub MVP - Integrate enhanced BH Clinician profiles - Develop structure, governance, and service model for same-day and real-time BH help	- Integrate ecosystem features as they roll out - Leverage partnerships to build, integrate, and release new resources and functionality
Perpetual Screening Ubiquitous, simplified screening across the clinical footprint	\$\$	Further research into potential partners and rollout options, understand clinician needs and potential data integrations for screening results	- Internal collaboration to create detailed design and prototype of pilot/MVP (Based on design of Hub) - Usability testing of experience design	- Create operational plans, marketing plans and training for AHN rollout - Develop and launch MVP
Peer Support—Anywhere Offline and online platform to connect people with peers for support.	\$\$\$	- Discussion with potential partners who bring scale and expertise in Peer Communities - Low-fidelity pilot of service for learning	- Internal collaboration to create detailed design and prototype of pilot/MVP (Based on design of Hub) - Usability testing of experience design	- Create operational and marketing plans - Staff for peer community organizer/curator - Develop and launch MVP and enhance feature set w/ additional rollouts
Navigational Pathways Platform to help the patient & provider understand what resources are out there, types of therapies, and next steps regarding BH care	\$\$\$	Leverage clinician input and existing LHS work on Anxiety and Depression solutions to understand potential areas for Pathway development	Define and create care pathways and continuums on most common or critical BH conditions	- Develop workstream for guideline auditing and begin to develop pathways for more complex cases and common cases for peds. - Refine and align educational tools to optimize patient experience and collect metrics
Culturally Competent Care Enhance the diversity of our clinical BH network and elevate our cultural competency as an organization.	\$	- Reconvene and expand D&I working group to represent diverse perspectives - Further research into partners, to understand clinician training needs and clinician pipeline challenges	- Internal collaboration to develop Culturally Competent Care training for AHN clinicians. - Pilot training with subset of clinicians - Begin work on longitudinal AHN BH workforce diversity plan	- Launch broader training within AHN and consider scale to clinical partners - Consider integration into existing technology platforms

Concepts for Parallel Workstreams or Future Roadmap

The remaining seven concepts can be considered for parallel development or future roadmap inclusion.

Content and Digital Learning Channels

Multichannel BH media

Bringing BH education, resources, and tools in a low-barrier format for everyone

Active BH Learning

Educate people on mental wellness through active engagement channels

Gamified BH

A multiplayer role-playing game with BH elements to build BH resiliency for youth (and others)

Analytics Foundational Capabilities

Identify Through Data

Leverage wearable, claims, and EMR data in novel ways for early outreach and intervention

Dedicated BH Research Group

A dedicated BH data analytics work group made up of a multidisciplinary team that continually rotates over time

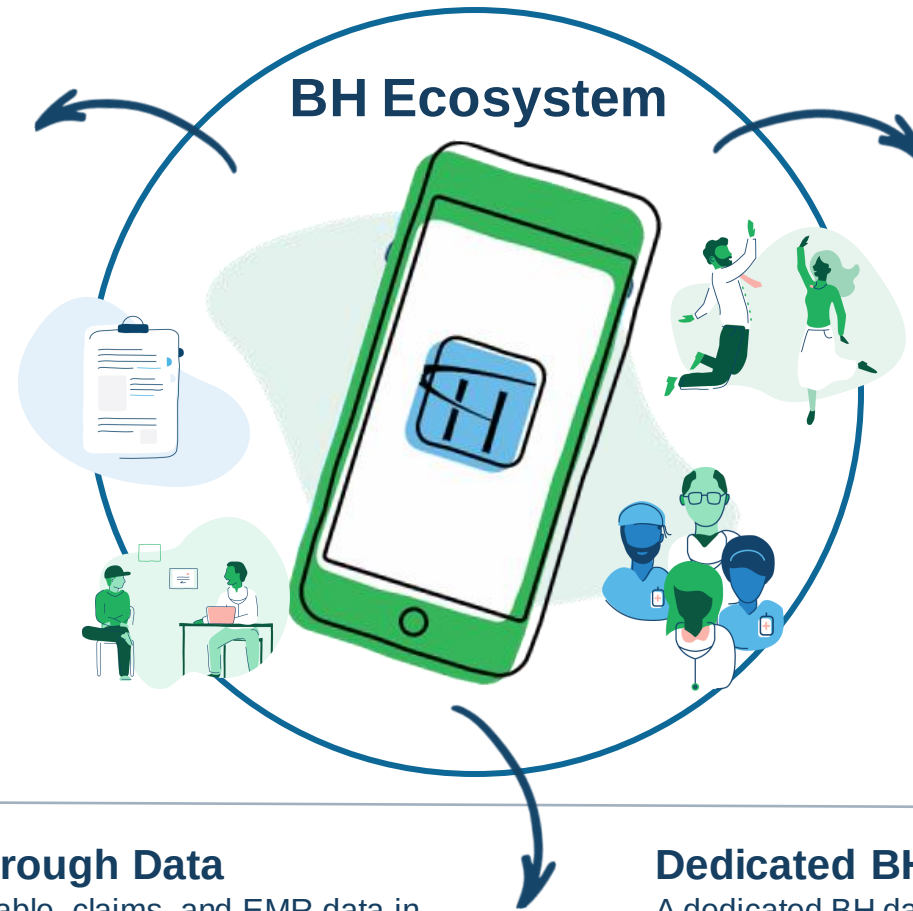
Individual and Community Engagement Programs

Personalized BH Box

Create a personalized “box” that provides tailored knowledge and resources

Provide a Support System

Ease individuals into trusting the system and seeking care to support them along their journey



Next Steps

- Syndicate with stakeholders and incorporate feedback
- Identify potential owners and collaborate on business case/solution approach for supported solutions
- Seek funding approval
- Integrate prioritized solutions into Living Health solution design process OR trigger Innovation Phase 3 to align with business owners, further define approaches, complete detailed design on solutions and test for customer/clinician acceptance.
- In addition, we can socialize remaining seven concepts for consideration with other stakeholder groups (Analytics, Marketing, SDOH, etc.?)

Opportunity Summaries and Solution Concept Details

Opp 1: Prevention:
Create awareness
and activate mental
resilience self-care
in at-risk populations

Key Takeaways: Prevention

Context and Support Information: With a delay between symptom onset and treatment that averages 8-10 years, we are waiting until patients show up with acute symptoms in the ED, or with complex, comorbid conditions before we address their underlying BH issues. Prevention of BH illness before it becomes acute, costly, and unmanageable will benefit our members and patients through improved outcomes and reduce costs of care.

Reactive vs. proactive care

Behavioral health illness prevention for clinicians is difficult, as patients often present with acute, co-morbid symptoms that have gone undiagnosed. Consequently, many BH clinicians feel that they are forced to be reactive rather than proactive when they provide care.

Self-care and resiliency

There are few clear methods of detection and engagement with the healthy population other than screening in a clinical setting. Without more efforts that engage healthy people to help them build resiliency and provide tools to stay healthy, we won't empower at-risk populations to engage in self-care before BH conditions start to develop.

Trauma is universal and hidden

Everyone experiences trauma. However it is not frequently discussed, understood, or acted upon until it has manifested into acute BH symptoms. These symptoms are often varied, subtle, and different for each person, making prevention and early intervention very difficult.

Support systems help with prevention

Several research respondents stated how lucky they felt to have family, friends — and even teachers — reach out to them when they noticed signs of BH their symptom onset. This support system is often the initial catalyst for a patient seeking out BH treatment. Additionally, patients who have received BH care often express a desire to help their loved ones find care, creating a preventive “pay it forward” effect.

Key Takeaways: Prevention

BH is not the same as physical health

Patients are not often aware of methods and tools to maintain their mental wellness, as they do with their physical health — until after engaging in care. This can make prevention of initial BH illness onset very difficult for people to self-address.

Empower patients with the right tools

Numerous research respondents indicated a strong desire for practical tools and methods to self-manage their mental wellness, often noting that such tools delivered a sense of empowerment and control over their own care. This helped them feel that preventing future BH illness was not only possible, but within their reach.

Solution Strategies: Prevention

Education and Awareness

Education and awareness campaigns that are targeted to specific communities that acknowledge their specific concerns, demystify behavioral health, and describe how mental wellness and resiliency training can help.

Self-management Tools

Create and deploy BH self-management or self-care tools that empower members to leverage their own resources in maintaining their mental wellness.

Build trust through Transparency

Make BH practices, methodologies, and benefits transparent to members in order to build trust and knowledge of behavioral health care.

Sense of community

Create groups of like-minded people with similar experiences that can help each other and lighten the load for BH clinicians. Build community spaces that will benefit multiple at-risk groups.

Concepts: Prevention

Innovation Concepts

Concept 1

Personalized BH Box

Create a personalized “box” that provides tailored knowledge and resources.

Concept 2

Active and Passive Learning

Educate people on mental wellness through active and passive channels.

Concept 3

Peer Support Anywhere

Offline and online platform to connect people with peers for support.

Concept 1: Personalized BH "Box"



Concept 1: Personalized BH "Box"

The Big Idea

Provide useful information and resources catered to people's personal needs for mental wellness and resiliency training.

Description

Patients observe and periodically document their mental health and are delivered customized wellness activities, tools, resources, and information based on their input and individual needs and goals. The activities, tools and information are designed through a collaboration of BH clinicians, life coaches, MI experts and designers. As part of someone's health plan, they can utilize this service without prior engagement with any clinicians. This service can provide a stepping stone to streamline access to care and future treatments. The concept can be further expanded to incorporate data tracking from wearable devices and other data points.

Unmet Needs It Satisfies

Provides patients the tools they need to maintain their total health and recognize their BH symptoms.

Key Features

- Pattern analysis of qualitative feedback
- Provide easy-to-digest activities and training for resiliency
- Tailored resources and communications
- Future integration with wearables

Key Audiences

- Existing members and patients
- Members who are willing to share personal wellness information

Key Stakeholders

- PCP's and BH clinicians
- Marketing and Digital Experience
- Outreach coordinators
- BH data analytics team

Concept 1: Personalized BH "Box"

Business Model

- Highmark-funded pilot as MVP
- Subscription service add-on for members and patients
- Future commercialization of scaled offering to other Blues and/or as EAP offering
- Partnership with vendor for further expansion and scale (i.e. MyStrength or Ginger)

Time considerations

- Initial pilot: ~ 6 months – 1 year
- Partnership + launch: 2 - 3 years
- Wearable Integration: 3 – 5 years

Success Measures

- Decrease in ED visits for BH emergencies
- Decreased costs of care: patients PMPM decrease over time
- Increase in brand loyalty, reputation and recognition
- Patient engagement in appropriate care at the right time

Benefits

- Prevention of behavioral health illness through early outreach
- Novel, patient-centered approach to prevention and early intervention
- De-stigmatizes behavioral health by making prevention for BH accessible, friendly, and personalized

Innovation Type

- Personalized Service and Customization
- Subscription model bundled into membership

Concept 1: Personalized BH "Box"

Next Steps

Pilot/Scale Considerations

- **6 months:** Pilot effectiveness of resiliency activities and information with custom information manually pushed from coaches (i.e. Relay 1:1)
- **w/in 1 year:** Expand pilot to web platform with natural language processing capabilities
- **w/in 3 years:** Expand to integrate data from wearables

Build/Buy/Partner Options

- Develop comprehensive inventory of resources and knowledge database
- Build digital platform with web and app access for users
- Build/partner with vendor to build natural language processing into platform
- Potential partnership with FitBit or BH vendor for integrated platform

Potential Implementation/Ownership Teams

Living Health Solutions
Integrated Marketing Campaign

Recommended Next Actions

Detailed Design Phase with Innovation and Living Health Solutions Curated Health Model Team in collaboration with Marketing and Digital Experience teams and BH clinicians

Concept 2: Active Learning



Concept 2: Active Learning

The Big Idea

Provide a platform to educate people on how to maintain their mental wellness through various channels and methods.

Description

A platform designed to deliver actionable BH resources and tools by actively engaging people, on their own time and terms. The platform provides members with the knowledge and skills to maintain their own mental health and wellness. People can engage with their health through webinars, workshops, classes and BH "workout" sessions on this platform.

Unmet Needs It Satisfies

Providing patients the resources to maintain their mental health and self-recognize BH symptoms

Key Features

- Platform for training, classes and resources by life coaches and therapists
- Comprehensive library of BH information and resources
- Flexible engagement options for key audiences
- Connect local communities to BH care

Key Audiences

- Existing members and patients
- Potential members

Key Stakeholders

- PCP's and BH clinicians
- Marketing and Digital Experience
- Outreach coordinators
- Life Coaches and instructors

Concept 2: Active Learning

Business Model

Highmark-funded, with content created using internal resources. Integrated into HM-AHN service offerings.

Time considerations

Initial pilot: ~ 6 months

Full Development: 1 - 2 years

Success Measures

- Patient engagement
- Increased usage and traffic through various channels
- User satisfaction ratings
- NPS and recognition scores

Benefits

- Prevention of BH illness in Highmark's patient population through awareness-building and self-activation
- Enables members to build both physical and digital support communities
- Positive brand image that embraces whole health
- Destigmatizes and demystifies BH
- Allows easy navigation of consolidated information in one place

Innovation Type

- Mastery – help customers obtain knowledge and skills
- Supplementary service to existing offerings
- Create User Communities/Support Systems

Concept 2: Active Learning

Next Steps

Pilot/Scale Considerations

- **6 months:** Pilot with remote classes in parallel with educational marketing through current social media channels
- **w/in 1 year:** Expand pilot by spreading to different entertainment mediums (i.e. podcasts) and pilot of physical classes
- **w/in 3 years:** Independent education platform for online learning

Build/Buy/Partner Options

- Gather and build the information/resource library
- Build curriculum and active engagement channel components
 - Source coaches/teachers
 - Integrate into AHN hospitals' or Direct Store class schedule system
 - Remote classes
- Collaborate with Marketing and Digital Experience team to build out social media and other communication channels
- Consider partnerships with national-scale organizations who are content creators. (Mental Health America, Henry Health)
- Potential to satisfy partially through partnership and integration with MyStrength and/or other digital solution providers.

Potential Implementation/Ownership Teams

Marketing and Digital Experience Team/Living Health Solutions

Recommended Next Actions

Design Development Phase with Innovation and Living Health Solutions
Curated Health Model Team in collaboration with Marketing and Digital Experience teams and BH clinicians

Concept 3: Peer Support Anywhere



Concept 3: Peer Support Anywhere

The Big Idea

Connect individuals with peers offline and/or online to better identify, prevent, and self-manage the onset or escalation of BH conditions.

Description

Many people do not have a support system of friends and family who can listen and understand what they're going through—and it is often hard for people to find and connect with therapists who share similar backgrounds and experiences. Through this peer connection platform, members can connect with each other, both online or offline, form small groups of likeminded peers to speak with, and create a safe space to have meaningful conversations about mental wellness.

Unmet Needs It Satisfies

Helps members connect with other people who share similar backgrounds, experiences, and conditions.

Key Features

- Platform to help facilitate communications and interactions
- Private channels for communication with peers

Key Audiences

- Existing members and patients

Key Stakeholders

- PCP's and BH clinicians
- Digital Experience Team
- Social Workers
- Peer Support Specialists

Concept 3: Peer Support Anywhere

Business Model

- Highmark self-funded pilot as initial MVP
- Freemium offering – free basic offering with premium add on features for additional cost
- Future integration with digital therapeutic partner for national scale (MyStrength, Axial, MAP Healthcare)

Time considerations

- Initial pilot: ~ 3-6 months
- Full Scale launch: 1 year
- Potential partnership with national scale: 1-2 year

Success Measures

- Engagement : # of people adopting and utilizing the platform over a period of time
- User satisfaction through survey
- Decreased costs of care: patients PMPM decrease over time

Benefits

- Reduces costs of care over time through early intervention and low-cost care
- Enables people to build the support groups they need so they feel listened to and care about
- Lower barriers to behavioral health access and information
- Builds trust with our members and patients

Innovation Type

- Crowdsourcing Process
- User-Community Support System Driven Service
- Community and Belonging Engagement
- Integrated Offering with AHN BH services and resources

Concept 3: Peer Support Anywhere

Next Steps

Pilot/Scale Considerations

- **6 months:** Pilot with select group of 100 members through manual selection and partnering or groupings
- **w/in 1 year:** Expand to integrate digital platform with built in chat/video chat functions
- **w/in 2 years:** Consider scale of community building function within the digital platform

Build/Buy/Partner Options

- Build operational flow for offline and online platform
 - The system for online communication
- Partner with local community organization and non-profits organizations
- Partner with digital vendor that also have peer-to-peer support integrated into their digital therapeutics or co-build into our future BH digital therapeutic vendor

Potential Implementation/Ownership Teams

Living Health Solutions
AHN Clinicians

Recommended Next Actions

- Detailed Design Phase with Innovation and Living Health Solutions Curated Health Model Team in collaboration with Marketing and Digital Experience Team and BH Clinicians

Opp 2: Bring New Approaches to Identifying Needs and Activating Care

Key Takeaways: Identifying Needs & Activating Care

Context and Supporting Information: To stay ahead of the curve, Highmark must focus on decreasing the escalating tide of behavioral health illness in our member population to curb the impact of higher condition-acuity and costs. This can be done through scaled interventions in the early identification and care activation spaces.

Self-Recognition is Difficult

Patients initially have a difficult time recognizing the onset of their own BH illness, as triggers and symptoms can often be subtle, or go unrecognized for long periods of time. However, once patients receive BH treatment, they often become highly attuned to their symptoms and triggers and the connection of their physical health with their mental health. This makes activating care easier.

Being Open to Getting Care

Many patients feel that they need to be "open" to BH care in order to fully reap its' benefits. Some may stay away from activating their care due to internalized biases, fear that they won't get the help they need, bad past experiences, or simply not being informed about what BH care can do for them.

Barriers to Activation

It is difficult for many patients to activate their care because of the multitude of barriers that prevent them from seeking help: prohibitive costs of both in- and out-of-network care, scarcity of available providers, wait times up to 6 months for a BH professional, lack of information about where to find BH help, clunky and/or non-existent referral processes, stigma, lack of trust, and clinician bias.

Expertise in Identification

While screening for BH issues is indeed an important tool in identifying BH conditions, many PCP's and non-BH clinicians do not act upon screening results because they do not feel they have the time, training, or expertise to handle and refer their patients appropriately.

Solution Strategies: Identifying Needs & Activating Care

Education & Awareness	Leverage the Moments that Matter	Support Self-Activation of BH Care	Enable Timely, Easy Access to BH Care
Education and awareness campaigns that are targeted to at-risk communities that acknowledge their specific concerns and describe how BH care can help.	Develop “identifiable BH triggering event” care protocols that allow us to conduct outreach and intervention efforts before BH conditions become acute.	Develop “self-screening” tools that allow members to understand and recognize BH symptoms and triggers in the privacy of their own environments,	Enhance transparency and practice methodology of providers in network for members to easily understand BH clinicians' availability, working methodologies, and other aspects of their practice to allow them to quickly activate care.

Concepts: Identifying Needs & Activating Care

Innovation Concepts

Concept 1

Identify Through Data

Leverage wearable, claims, and EMR data in novel ways for early outreach and intervention

Concept 2

Global Screening

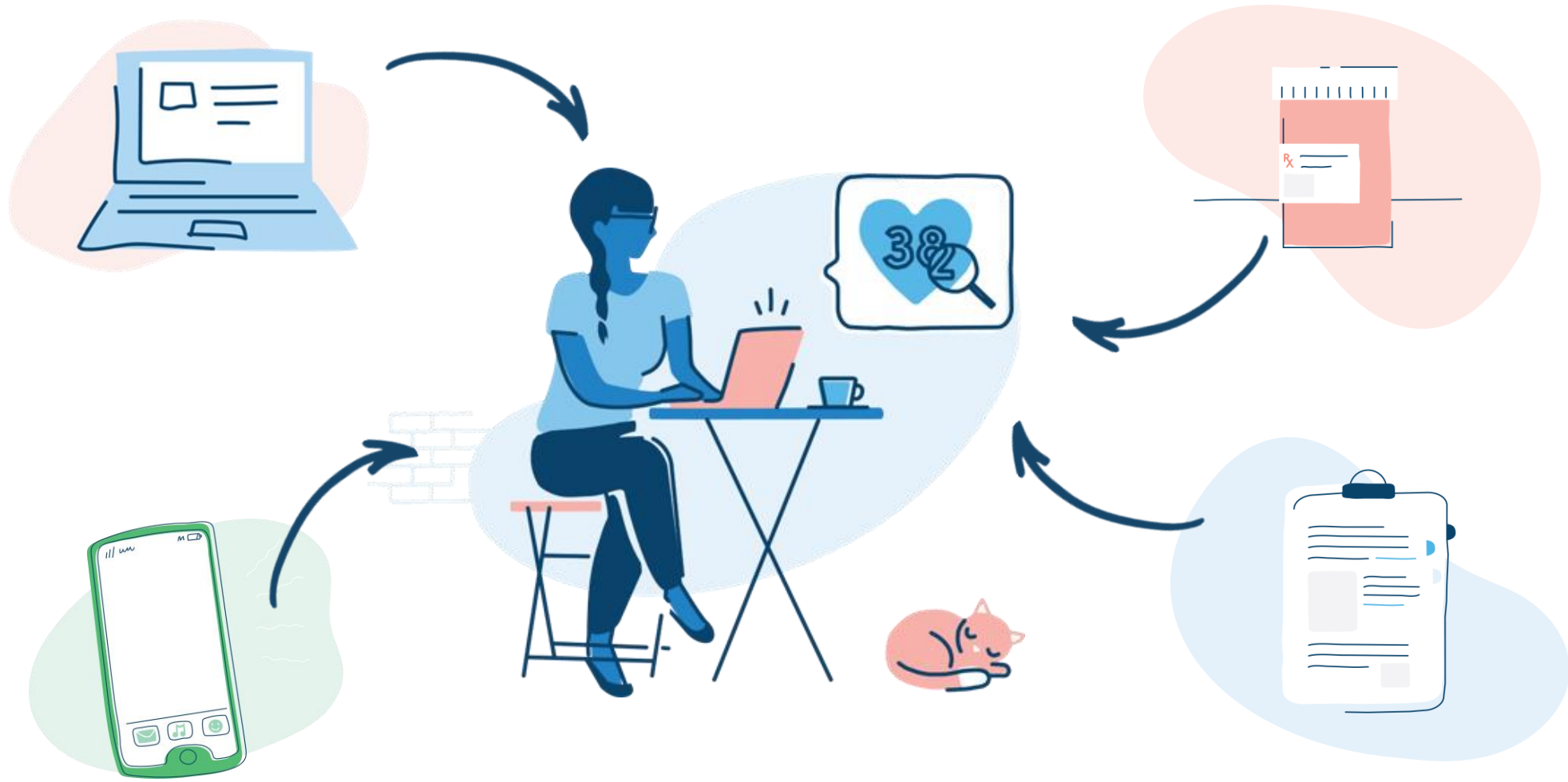
Ubiquitous, simplified screening across the clinical footprint

Concept 3

BH Care Hub

Streamline care activation through transparent, simple information and tools

Concept 1: Identify Through Data



Concept 1: Identify Through Data

The Big Idea

Identify important moments when intervention may be necessary through data analysis

Description

Description: we can identify — and intervene — at critical moments when BH care may be needed through data analysis and appropriate outreach. Moments such as change in marital status, loss of employment, and family changes can be identified through claims and EMR data. And physical conditions that may trigger BH illness such as a chronic disease or cancer diagnosis, lack of physical activity, changed sleep patterns, or inconsistent heart rates may all indicate that help could be needed. By leveraging and analyzing available data on behavioral and medical patterns, we can proactively provide education and outreach to our members. Outreach can be activated digitally through secure messaging in the patient's EMR/patient portal, or directly through a plan “sherpa” or guide, or through the member's PCP. Information such as recommended next steps, information about disease states that affect BH, and referrals will be shared with both patient and PCP in the outreach effort.

Unmet Needs It Satisfies

Helping patients identify impending BH issues before they become acute

Key Features

- Uses available data sources to identify BH “triggers” that require outreach
- Pattern analysis of EMR, claims, Member listening system, partner, FitBit and other wearables data
- Tailored communications and outreach
- Information on next steps
- Education on triggers for BH illness

Key Audiences

- Existing members across AHN-HM footprint

Key Stakeholders

- BH data analytics team
- PCP's and BH clinicians
- Outreach coordinators

Concept 1: Identify Through Data

Business Model

Navigational tools aimed at increase education can be licensed for national use.

Time considerations

Year 1-2: Work with in-house analysis teams to pull existing data from claims, Member Listening System, and partner data (Sharecare, Livongo, etc) to identify patterns, behaviors and disease-states that increase risk of BH complications.

Year 2-4: Integrate with Living Health Dynamic Platform to build robust algorithms that can identify more complex behavior patterns, and alert appropriate clinical and social teams to take interventional action on patient needs

Year 5+: Refine and license BH algorithms

Success Measures

- Customer engagement (med adherence, follow-up, etc.)
- Increased Highmark membership
- Demand for scaling the solution regionally or nationally

Benefits

- Highmark Health as a thought leader in behavioral health transitional care and identification
- Driving customers to Highmark membership and services
- Reduced PMPM over the long-term for its membership
- Prevention of acute BH illness in Highmark's patient population through early identification and outreach

Innovation Type

- Delivers content that is metrics-based and clinically-focused
- Captures patients in the system before they require intensive IP or IOP therapies.

Concept 1: Identify Through Data

Next Steps

Pilot / Scale Considerations

- w/in 6 mos: Collaborate and build partner clinical team to help create discharge protocols and identify metrics that should be analyzed in the system
- w/in 1 yr: Initial internal build to create data analytics structure, conduct feasibility study, and pilot initial identification system; align with technology & integration teams to collect data and analyze claims and EMR for success measures; develop updated systems listing and discharge planning paperwork for activation in the community; and develop communication and information protocols based on identified data flags, and pilot initial member communication / outreach service

Build/Buy/Partner Options

- Collaborate with external clinical partners on best practices in the post-acute care setting
- Leverage any short-term solution development vendors/partners already being targeted by LH Curated Health Model Team for short-term impact
- Consider how this may be delivered or scaled in collaboration with analytics engine partner
- Consider wearables partnership

Potential Implementation/Ownership Teams

- Enterprise Analytics
- Living Health Solutions
- AHN and Highmark BH Clinical Stakeholders

Recommended Next Actions

- Detailed Design Phase with Living Health Solutions in collaboration with BH Clinicians, and AHN Transitional Care Planners

Concept 2: Global Screening



Concept 2: Global Screening

The Big Idea

Simplified screening tools leveraged across our entire clinical footprint that allow non-BH clinicians to easily recognize and address BH illness onset and provide opportunities for patients to self-screen in the privacy of their own home.

Description

Streamline BH screening to a plain-language questionnaire that asks simplified questions to help patients and non-BH clinicians identify and address the onset of BH symptoms. Questions will be asked in a way that minimizes engagement, prompting members with a quick, 2-question check-in. If the patient's responses signal that they need help, they will be given more comprehensive screening. The questionnaire data will be shared with both patient and their non-BH clinician/PCP to recommend programs and interventions, and guide referrals to an appropriate BH professional as needed. The questionnaire will be made available across the AHN footprint — in person, and as a digital pre-appointment exercise for patients.

Unmet Needs It Satisfies

Helps non-BH clinicians — and their patients— recognize when BH symptoms need to be addressed. Takes the time and expertise gap burdens off of clinicians doing BH screenings and empowers patients.

Key Features

- Simplified, qualitative questionnaire that can be completed digitally at home, prior to appointments
- Provides real-time info for patients and providers on available next steps, based on symptoms
- Available to all non-BH clinicians and practices

Key Audiences

- All AHN-Highmark members
- Non-BH clinicians

Key Stakeholders

- PCP's and non-BH clinicians

Concept 2: Global Screening

Business Model

Highmark-AHN funded
IP of questionnaire and business model can be monetized
for national scale

Time considerations

AHN footprint pilot: 6mo-1 year
Scale to partners: 1-2 years
Scale to Highmark members: 2 years +

Success Measures

- Increased BH screening across AHN's footprint: # of screenings completed
- Increased # of referrals made to in-network BH clinicians
- Decreased costs of care: reduction of PMPM

Benefits

- Becomes the scalable structure for Highmark-AHN's early intervention efforts
- Streamlined BH screening efforts can reduce costs of care over time
- Can become monetized and scaled nationally

Innovation Type

- New service model for offering scalable, progressive BH screening

Concept 2: Global Screening

Next Steps

Pilot/Scale Considerations

- **w/in 1yr:** Initial internal development of screening content, user testing, and initial pilot roll-out for select AHN sites
- **w/in 2 yr:** Build infrastructure and staffing for post-screening outreach services
- **w/in 3yr:** Scale content modules / IP nationally through partnership with Epic

Build/Buy/Partner Options

- Leverage any short-term solution development vendors/partners already being targeted by LH Curated Health Model Team for short-term impact
- Potential partnership with Epic/MyChart for scaling and monetizing screening modules beyond HM footprint

Potential Implementation/Ownership Teams

- Enterprise Analytics
- Living Health Solutions
- AHN and Highmark BH Clinical Stakeholders

Recommended Next Actions

- Detailed Design Phase with Innovation and Living Health Solutions Team in collaboration with Analytics, and AHN/Highmark Clinical Stakeholders

Concept 3: BH Care Hub



Concept 3: BH Care Hub

The Big Idea

A digital resource hub that makes accessing and maintaining BH care and resources simple, straightforward, and non-intimidating.

Description

The BH Hub is a digital platform for those seeking help, connection, information, and resources for their BH. It will provide plain-language information, relevant content and resources, and provide actionable next steps for people seeking to activate and maintain their BH. The Hub will host an up-to-date guide on available in-network BH providers and contain curated resources, tools, and real-time services for BH and wellness. The provider guide will include an individual's specialty, areas of focus and working style to help members understand if they will be an appropriate fit and allow both appointment scheduling and same-day telehealth visits. Additionally, the platform will host digital wellness solutions, peer and community group solutions, and new content as it becomes available within Highmark's BH solutions portfolio.

Key Audiences

- People seeking and maintain their BH care
- In-network BH clinicians

Key Stakeholders

- In-network BH clinicians

Unmet Needs It Satisfies

Allows people seeking help to easily activate BH care in a safe, non-threatening space that gives them the information they need to make informed decisions about their care.

Key Features

- Up-to-date information and scheduling with in-network providers
- Available BH providers for real-time telehealth
- Curated BH wellness information, resources, and tools, including peer groups and community services
- Information on actionable next steps and care plans

Concept 3: BH Care Hub

Business Model

Partnership with Amwell / MyStrength / Aunt Bertha

Time considerations

2–3 years

Success Measures

- Patient engagement: number of patients referred to HM-AHN's BH services
- Increased # of referrals made to in-network or covered virtual BH services
- Decreased costs of care for participating patients: reduction of PMPM

Benefits

- Increase plan membership
- Increase patient engagement and satisfaction through providing streamlined access to BH care

Innovation Type

New service model for patient access

Concept 3: BH Care Hub

Next Steps

Pilot/Scale Considerations

- **w/in 1yr:** Internal research, design, and working prototype of hub for pilot/MVP
- **w/in 1yr:** develop structure, governance, and service model for same-day and real-time BH help
- **w/in 2yr:** Partnership to build, integrate, and release full functionality

Build/Buy/Partner Options

- As first step to virtualize clinical capacity for real-time or same-day visits, work with Transformed Practice BHCs and mature the "pooling model" already in place as a reaction to COVID.
- Consider potential partners for co-creation of Digital Hub, including Amwell / MyStrength / Aunt Bertha
- Consider how this could be built into the Integrated Experience digital solutions

Potential Implementation/Ownership Teams

Living Health Solutions

CX and Digital Experience

Living Health Digital Experience Stakeholders

Recommended Next Actions

- Detailed Design Phase with Innovation and Living Health Solutions Curated Health Model Team in collaboration with Integrated Experience Layer/Digital Experience Team

Opp 3: Activate the Entire Caregiver Community

(non-BH clinicians, unpaid caregivers, local communities)

Key Takeaways: Activate the entire caregiver community

Context and Supporting Information: There is a shortage of BH clinicians, and BH need is outpacing the ability of the system to provide care. One way to help address this need is to activate the larger caregiver community, including non-BH clinicians, unpaid caregivers, and local communities.

Lack of time to practice BH

Non-BH clinicians, like all clinicians, have too much on their plate and struggle to handle their patient loads. As a result, many non-BH clinicians express that BH care is out of their scope of work. Challenges with referrals and BH resources provide a further disincentive to engage with BH care.

Lack of resources

BH education, tools, and resources exist for the general population. However, they are often not very accessible, and it is hard to know which are reliable. Caregivers often feel ill-equipped to engage with these resources in order to better care for their loved ones.

Stigma barriers to engagement

The stigma around behavioral health can be a disincentive for all types of caregivers to engage in BH discussions. This can be particularly challenging for certain communities or demographics. Normalizing conversations around BH is critical if caregivers and care recipients are going to benefit from behavioral health as a society.

Complexity and confusion

Understanding behavioral health is complex enough, but navigating it is both complex and confusing. Customers complain the system is “written in a foreign language” and “not there to help them.” Frustration with navigating care leads many to give up trying, delaying seeking care and prolonging their care, potentially leading to additional health consequences.

Solution Strategies: **Activate the entire caregiver community**

Build trust

Leverage Centers of Trust to engage with at-risk communities and begin to build trust so that these communities will engage with BH care. Leveraging these centers also break down stigma.

Lifestyle fit

Caregivers are particularly busy people. Meet people where they are in their daily lives. Engage with them through channels they already use and help the message stand out from everything else they get bombarded with.

Culture fit

Reduce stigma by making BH a part of the everyday culture and help to normalize BH conversation to reduce confusion, foster discussion, and help build connections for caregivers and those seeking care.

Easy tools

Provide tools and resources in a way that is easy to understand and is actionable. Help people understand this information within the context of their own experiences. Make it easy for them to self-navigate to find what they need to find and/or provide BH care.

Concepts: **Activate the entire caregiver community**

Innovation Concepts

Concept 1

Multichannel BH media

Bringing BH education, resources, and tools in a low-barrier format for everyone

Concept 2

Gamified BH

A multiplayer role-playing game with BH elements to build BH resiliency for youth and others

Concept 1: Multichannel BH media



A multichannel media venture that positions Highmark Health as a thought leader in societal issues (i.e. BH) and delivers low-barrier, actionable, and “fun” BH content to the masses, particularly to caregivers and communities.

The solution will position Highmark Health as a thought leader regionally and nationally and will drive increased membership.

Concept 1: Multichannel BH media

The Big Idea

Everyone needs to engage with behavioral health, particularly caregivers. However, reaching caregivers is not always easy due to their busy lives. It is critical that we reach them where they are, in a format that fits their lifestyle, but also in a way that builds a common understanding and BH “language”, so that we can all work together as caregivers (as individuals and as clinicians) to address the behavioral health crisis that is before us.

Description

A multichannel media service that delivers bite-sized yet impactful BH content in a way that is engaging, educational, and actionable. Types of content could include having local leaders talk about BH in their communities; entertainment segments that discuss BH within the context of pop culture; a national BH-start-up spotlight hosting start-up CEOs; AHN BH clinician interviews and BH discussions; 10-min BH resiliency exercises; etc. As this platform grows, it could create separate “channels” for clinicians and individuals.

Unmet Needs It Satisfies

- Reducing stigma
- Equipping the masses with BH education and ability to navigate BH needs
- Addressing BH needs earlier to prevent escalation

Key Features

- Multimedia, multichannel format
- Engaging content via multiple societal lenses

Key Audiences

- All adults
- Possibly leverage Highmark employers
- HH Provider Partners and larger Blues audience

Key Stakeholders

- AHN clinicians (BH and non-BH)
- Highmark Health marketing
- Local community leaders
- Influencers and media content creators

Concept 1: Multichannel BH media

Business Model

A freemium service. Potential buy-ups include additional content, personalized resourcing, etc.

Time considerations

In the next 1–2 years launch a microsite with targeted content and then over the next 2–4 years build the partnerships and additional content.

Success Measures

- Customer engagement (sign-ups, churn, etc.)
- Increased Highmark membership
- Demand for scaling the solution regionally or nationally

Benefits

- Highmark Health as a thought leader in behavioral health and societal issues
- Driving customers to Highmark membership and services
- Reduced PMPM over the long-term for its membership

Innovation Type

- Freemium model
- Pulling in community leaders and regional/national influencers to produce engaging content
- Delivers content unlike anything else out there—engaging and entertaining content to eliminate stigma and provide BH tools & education
- Multichannel format to deliver content where, how, and when people need it to fit their lifestyle

Concept 1: Multichannel BH media

Next Steps

- **Key Stakeholders for buy-in:** AHN, Marketing
- **Recommended Next actions:**
 - Conduct internal review of capabilities and benchmark best-practices of analogous offerings
 - Determine where the product lives – it shouldn't have the “baggage” of an insurance company but should have a clear line to Highmark Health for thought leadership development
 - Leverage Highmark Health and AHN resources to build a microsite with limited content. Determine what the core content would be and what specific audiences would be targeted
 - Leverage the success of the microsite to build new capabilities and recruit strategic partnerships to build capabilities, content, and channels
- **Pilot/scale considerations:** Build small microsite with more limited content and target a defined, easy to reach population to start. Add content and capabilities when ready. Could scale to Provider Partner areas and the Blues.
- **Resources needed:** Clinical expertise & content creation; multimedia development; marketing expertise; influencers
- **Build/Buy/Partner:** Partner w/ multichannel media creators
- **Potential implementation:** Owned by Marketing but partnered with AHN clinicians

Concept 2: Gamified BH



A multiplayer life-simulation game which incorporates behavioral health themes in order to help youth develop BH literacy and skills.



Animal Crossing New Horizons (by Nintendo) is the second-best selling game of 2020. It is a non-linear life simulation game played in real-time.
animal-crossing.com

Concept 2: Gamified BH

The Big Idea

Half of mental health conditions develop by age 14 and $\frac{3}{4}$ by age 25. The most influential caregivers for youth are parents, but parents are often not equipped to address these issues with their children. Historically, gaming (sports, chess, etc.) has been a proven avenue to help children develop critical life skills. Gamifying BH literacy and skill development within the context of games they already play, may be a means to help children develop BH resiliency and reduce the onset of these conditions.

Description

A life simulation game that incorporates BH elements. These could be around conflict resolution, decision making, mindfulness, and other BH themes that would help children develop BH skills. The game could encourage children to engage with their parents about what they are learning. Adolescent and teen versions of the game could utilize a peer-to-peer function to build BH resiliency and act as peer-support. Educational approaches could be assessed through trackable gaming elements.

Unmet Needs It Satisfies

- Reduces stigma
- Builds BH resiliency for an age group that is largely unaddressed

Key Features

- BH resiliency skill development
- Gamification

Key Audiences

- Children
- Adolescents and teens

Key Stakeholders

- AHN BH clinicians (content development)
- Game designers
- Parents
- Children

Concept 2: Gamified BH

Business Model

A partnership involving BH content creation and game development. The product is a downloadable app. Highmark insurance members may receive a discount. The game could be piloted through AHN's CHILL Project.

Time considerations

2–3 years to develop and launch the game

Success Measures

- # of purchases/downloads
- Customer engagement (sign-ups, churn, etc.)
- Improved health and wellness for this age group
- Awards/recognition for Highmark Health in addressing youth behavioral health

Benefits

- Highmark Health as a thought leader and innovator in BH care for children and adolescents
- Improved health outcomes for this age group
- AHN as a destination for BH clinicians
- Development of new BH educational and skill development strategies

Innovation Type

- An innovative channel of bringing BH education and skill development through mobile gaming
- New processes and strategies for BH education and skill development could be developed
- The product could be diversified into a franchise that addresses additional populations (college students, young adults, retirees, seniors)

Concept 2: Gamified BH

Next Steps

- **Key Stakeholders for buy-in:** AHN, Center for Traumatic Stress in Children and Adolescents, Marketing (Digital)
- **Recommended Next actions:**
 - Conduct industry analysis and benchmarking on best practices and identify strategic partner(s)
 - Investigate 2 possible development approaches:
 - Look for potential game development partners and create a game from scratch. Initial suggestions for partners to explore include Schell Games and CMU's ETC. CMU's psychology department may be an interested partner for content creation. In such cases, a student capstone project could be a way to determine high-level scope and requirements.
 - Find a developer of an age- and content-appropriate game and seek to develop a next-generation version of the game that incorporates BH content.
- **Pilot/scale considerations:** Create simpler version, perhaps even in comic-book style, to place in pediatrics waiting rooms for children to interact with and gauge their response. The CHILL Project could be a perfect site to pilot.
- **Resources needed:** Game development partner, BH clinicians for content creation, marketing
- **Build/Buy/Partner:** Partner. AHN contributes content. The game developer builds, sells, and manages the game.
- **Potential implementation:** Determine the structure of the partnership. Potentially, AHN co-develops the content but the game developer builds and sells the game. AHN/Highmark Health would get co-branding.

Opp 4: Align continuum of needs with right level of care

Key Takeaways: **Align needs with right levels of care**

Context and Supporting Information: Today, the BH landscape of treatment is very subjective and not as metric or clinician-driven as the other medical sub-specialties. When behavioral health conditions are diagnosed, community engagement with treatment and an understanding of the entire health system is needed to be able to properly manage disease states—as they are chronic and fluid.

Inconsistent Treatment

Providers want to be empowered to know exactly what to do when a patient presents with a suspected BH condition, regardless of complexity. Additionally, BH therapies lack a strong fidelity in treatment and alignment on care and because of these different levels of care.

Data is Not Leveraged

Current available data is not used to promote and track good clinical therapies and patient outcomes, so symptoms go undiagnosed and unaddressed for a long time and then patients wind up in a high acuity setting such as the ED or IOP due to the lack of structure and outpatient services.

Education Barriers to Engagement

Patients are often unable to identify new or changes in BH symptoms due to a variety of stigmas and subtleties that mask changes as other causes or conditions. Patients are also often unaware of the variety of therapies available to them and do not have the vocabulary or understanding to ask about options.

Complex System to Navigate

Providers and patients do not have a clear understanding of the facilities, treatments, and transitional care plans that are available to the population. Therefore, care is very subjective and often determined by opinion and word of mouth, which is unlike other medical sub-specialties which are clinician and data led.

Solution Strategies: **Align needs with right levels of care**

Recommendations:

Care Pathway Integration

Caregivers are particularly busy people. Meet people where they are at in their daily lives. Engage with them through channels they already use and help the message stand out from everything else they get bombarded with.

System Navigational Guides

Leverage Centers of Trust to engage with at-risk communities and begin to build trust so that these communities will engage with BH care. Leveraging these centers also break down stigma.

Educational Tools

Reduce stigma by making BH a part of the everyday culture and help to normalize BH conversation to reduce confusion, foster discussion, and help build connections for caregivers and those seeking care.

Concepts: **Align needs with right levels of care**

Innovation Concepts

Concept 1

Navigational Pathways Platform to help the patient & clinicians understand resources, types of therapies, and next steps regarding BH care, with a foundation in robust data analysis and clinically proven care.

Concept 1: Navigational Pathways



A cross-system digital “Sherpa” that educates and informs along the care continuum. The tools position Highmark Health as a leader in breaking down the complexities of the health system that will enable greater engagement through care transitions, and deliver low-barrier, actionable, and clear next steps to patients, caregivers and providers when they need it most.

Concept 1: Navigational Pathways

The Big Idea

Mental health treatment is not as standardized as other medical specialties. There are many different treatment options that are available for conditions and a lack of clarity on what therapies would best work in the interest of the patient and their caregiver community. By providing a set of both clinical and system guides, we can remove a very complex barrier to care.

Description

Providers would get a set of metrics-based clinical treatment pathways that explore the various treatment options. This will remove barriers to providing proper care when patients present with a BH condition (especially when they are in a non-BH clinical setting). Patients and providers would also get a clinical continuum pathway that explains what symptoms are present and treatments that are available in acute state vs management state. This will help guide and educate the population to recognize and transition from levels of care.

Unmet Needs It Satisfies

- Standardizes treatment
- Reduces stigma through mis-education
- Increases engagement in the disease state

Key Features

- BH Care Pathway Management
- BH-Specific Discharge Plans
- BH Condition and Continuum Management Tools

Key Audiences

- BH Clinicians
- Non-BH Clinicians
- All BH patients and their caregiver communities

Key Stakeholders

- AHN BH clinicians (content development)
- HH CTO
- Living Health Dynamic Platform
- Living Health Curated Health Model Team

Concept 1: Navigational Pathways

Business Model

A partnership involving BH content creation and clinical strategic planning.

Time considerations

2 years to work with clinical, data, and CX teams to define care pathways on most common BH conditions and develop the disease-specific tools and literature

Success Measures

- # pathways created
- # Discharge transition plans updated
 - # of BH conditions getting treatment as a result of chronic condition acute events
- Customer engagement (downloads, feedback, churn, etc.)
- Improved health and wellness

Benefits

- Putting a clear guide around what the typical course of treatment looks like (including therapeutic options, medication analysis & explanations will help to enable the patient to be more engaged in their care and expand their knowledge base to ask the right questions and understanding what the spectrum of their care looks like
- Treatment Protocols for both BH and non-BH clinicians to guide therapies and provide metric-based treatment planning and standardized protocols which will enable clearer and differentiated treatment options while providing the patient with a defined structure to their therapy and treatment
- Addressing and engaging patient with BH conditions that are supplementary to chronic conditions to improve health outcomes

Innovation Type

- Total health experience management by providing an innovative channel of care alignment across non-BH clinicians
- Experience simplification by taking the guessing out of care management
- Integrated offering of BH support in the chronic and acute disease management setting

Concept 1: Navigational Pathways

Next Steps

Pilot/Scale Considerations

Year 1: Work with key stakeholders to define and create care pathways and continuums on most common BH conditions

Year 2: Refine and review educational tools to optimize patient experience and collect metrics on success measures

Year 2-3: License care pathways and journeys

Build/Buy/Partner Options

- Collaborate with external clinical partners on care pathways
- Partner with external vendors to develop continuum journey in a tangible manner
- Build out solution through LH Dynamic Platform

Potential Implementation/Ownership Teams

Living Health Solutions, Highmark Health BH Clinical Leads, AHN BH Clinical Leads

Recommended Next Actions

- Detailed Design Phase with Living Health Solutions Team in collaboration with BH Clinicians, and HP vendors

Opp 5: Be a leader in BH clinical insights through research

Key Takeaways: BH clinical insights through research

Context and Supporting Information: BH is a “young” science and much needs to be done to improve the health of those with BH conditions. One way to address this is to dedicate resources to developing, testing, and validating evidence-based treatments and methods. The added benefit of this is to position Highmark Health & AHN as a leader in BH and help grow AHN’s reputation & capabilities in the BH space.

Still a young science

The science of behavioral health is still in many ways a young science and lags behind that of physical health. Concerted efforts are needed to better understand BH conditions, which can often have overlapping causes and symptoms.

Compounding Complexity

People with behavioral health conditions often have physical health conditions and vice versa. These confounding factors can complicate the treatment of BH conditions. The typically long treatment and recovery times can make the benefit of recovery or lack of recovery less clear.

Evidence-based practices

Treatments for BH conditions can vary widely by clinician, which slows the understanding of what treatments lead to better outcomes. There is a need for the development of and adherence to evidence-based practices in BH care.

Solution Strategies: BH clinical insights through research

Blended data

As a blended health organization, Highmark Health is uniquely positioned and should take full advantage of its available data to better understand BH practices and their health outcomes

Data analytics

Data analytics is a cost-effective research avenue and should be leveraged as a primary means of BH research. Due to the complexity of BH, data analytics may be the only mechanism able to fully illuminate and make sense of the complexity.

Multidisciplinary

Leverage the collective expertise of clinicians, scientists, and data scientists. No one group will have the full picture in BH. It will take a collaborative to truly develop the kinds of clinical insights that will set AHN and Highmark Health as a leader in BH.

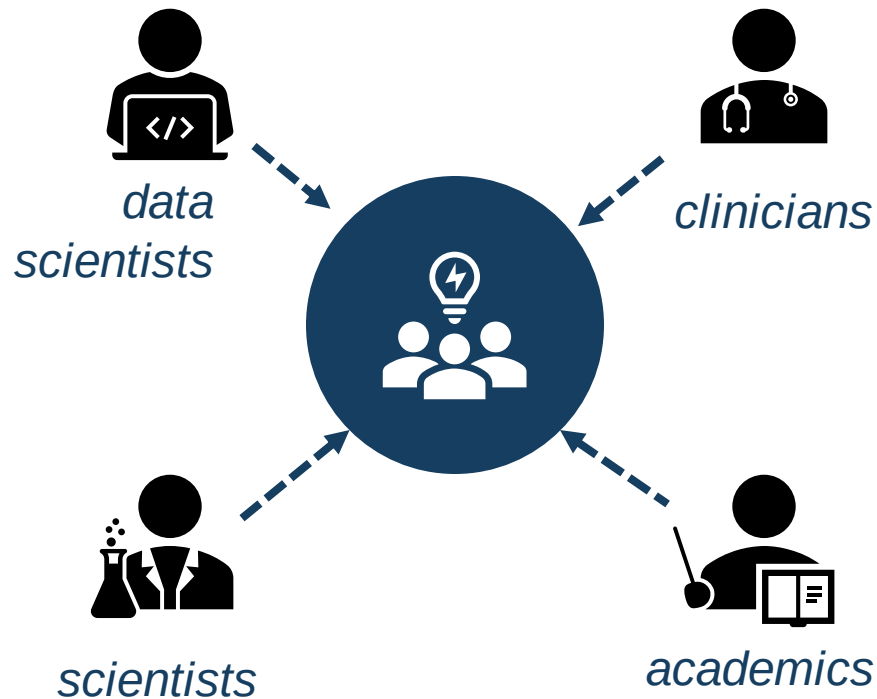
Concepts: BH clinical insights through research

Innovation Concepts

Concept 1

A dedicated BH data analytics work group made up of a multidisciplinary team that continually rotates over time

Concept 1: BH data analytics work group



A working group made up of diverse practitioners focused on improving the practice and outcomes in BH care.

BH leaders set the larger BH strategies and priorities. The working group asks the questions, performs the research, and develops the insights.

The working group has permanent members and rotating members. The rotational component allows for fresh ideas and techniques and then seeds these members back into the larger enterprise after completing the rotation.

Concept 1: BH data analytics work group

The Big Idea

Highmark Health has BH claims, outcomes, and other data that is not being fully leveraged. Clinicians are not being empowered to engage with data scientists to leverage data sources to further their practices. This idea creates a dedicated, multidisciplinary working group to develop BH clinical insights that will improve BH care and reduce costs. BH leaders will set the larger BH strategies and priorities for this working group.

Description

The working group will be made of clinicians, scientists, data scientists, and possibly academic research partners. They will be empowered by leadership to ask the right research questions, perform the research, and develop the insights. The working group has permanent members and rotating members. The rotational component allows for fresh ideas and techniques and then seeds these members back into the larger enterprise after completing the rotation.

Unmet Needs It Satisfies

- Evidence-based practices
- Reduced costs in BH care

Key Features

- Dedicated working group
- Data analytics

Key Audiences

- AHN BH clinicians
- Allegheny Research Institute
- Research partners (CMU, Penn St. Health)

Key Stakeholders

- AHN BH clinicians
- Allegheny Research Institute
- Academic research partners
- Living Health Solutions
- Non-BH clinicians

Concept 1: BH data analytics work group

Business Model

A group/department within Highmark Health. Processes developed and data insights could be commercialized. Commercialization opportunities through data sharing and analysis with the Blues Association.

Time considerations

Within 6-12 months this group could be up and operating.

Success Measures

- Insights leading to improved practices that lead to more efficient and effective BH care
- Publications and disseminated materials to the public that lead to Highmark Health being recognized as a thought leader and practice leader in BH

Benefits

- Highmark Health as a thought leader in behavioral health
- AHN seen as a leader in BH practice and development of evidence-based practices
- Reduced PMPM

Innovation Type

- A unique structure to recruit, train, and disseminate talent
- Unique and proprietary process and algorithm development
- A network designed to ask & answer questions and implement insights

Concept 1: BH data analytics work group

Next Steps

- **Key Stakeholders for buy-in:** AHN, Data analytics, and Allegheny Research Institute
- **Recommended Next actions:**
 - Get buy-in from leadership and determine ownership and operating structure
 - Determine space and resource needs
 - Work out rotational aspects with departments and HR
- **Pilot/scale considerations:** Create a 1-year ad-hoc workgroup made up of a small team of AHN clinicians and data scientists to research 1-2 problems, implement the clinical insights, and track outcomes. Partial/full FTE requirements are TBD based on the problems to be solved. Use outcomes and lesson learned to scale the workgroup.
- **Resources needed:** Clinical expertise; data analytics; software tools; clinical research; academic research partners
- **Build/Buy/Partner:** Build using Enterprise resources
- **Potential implementation:** Owned by Highmark Health

Opp 6: Cultural Competence in BH Care

Context: Cultural Competence in BH Care

In order to best serve every one of our members and patients, Highmark Health must work to meet the Behavioral Health needs of the individuals and communities that are currently underserved, **in particular, the African American population.**

Black people are burdened with a set of unique challenges that increase the severity of behavioral health issues, while the barriers to accessing and engaging in necessary BH care, that are experienced by the general population, are felt more significantly by Black people.

With a damaged relationship between these communities and the Behavioral Health system, as well as the healthcare system in general, there are low rates of accessing care. The same issues are seen at the individual scale between patients and providers. **The care that we deliver was not designed to meet the unique needs of these communities**, so even when the barriers to access are lowered, the quality and effectiveness of the care delivered suffers.

If we can solve for these problems, we can apply principles we learn to serve everyone better. There is also an opportunity to leverage the disproportionately high influence that Black communities have on American society relative to their portion of the general population.

Key Takeaways: Cultural Competence in BH Care

Significant Trauma

Individuals in the Black community are exposed to higher levels of trauma than the general population. Examples of this includes but not limited to being a witness to violent crime, the incarceration of loved ones, over policing and highly publicized examples of the deaths of Black people. This also includes the collective trauma accrued from years of experiencing constant microaggressions.

SDOH

Individuals' mental health is disproportionately impacted by negative Social Determinants of Health. Communities that lack access to quality education, housing, and employment opportunities are disproportionately Black. This is on top of dealing with lower average incomes and higher average rates of uninsured which increases the already steep financial barrier.

Unmet Needs

A lack of culturally competent BH providers and treatments makes it harder to meet the needs of communities. Individuals feel that BH care is not designed for them and therefore won't be worth the time. The lack of trust makes it impossible to build the necessary patient-provider relationship which leaves the patient feeling disconnected from their care and leaving it ineffective.

Stigma and Distrust

Intense stigma along with a historic distrust of the healthcare system decreases the likelihood that individuals will engage in BH care. This stigma comes from people and communities and is aimed at providers, patients and people living with BH conditions. This may prevent people from "opening up" during treatment or from ever seeking care at all or admitting that it's needed and that they need help in the first place.

Solution Strategies: Cultural Competence in BH Care

Deliver Culturally Competent Care

We need to deliver the type of care that meets the unique needs of the African American individuals and communities. We can do this by elevating the capabilities of our workforce as well as diversifying the clinicians that we hire.

Foster Support and Engagement

Care doesn't stop after a therapy session ends. It extends to the community. We need to be there to foster a supporting environment that continues to positive effects of the BH care we directly deliver. We also need to increase engagement with the importance of the BH system.

Establish Trust and Reduce Stigma

In Black communities, trust of the BH system is lower and stigma is higher, than in the general population. We need to work with our providers, and with community establishments and respected leaders in order to change this dynamic so that these barriers to receiving care can be reduced.

Impact Trauma and SDOH

Black communities struggle disproportionately with trauma and negative social determinants of health. This increases the severity and complexity of mental health conditions. By better serving these populations with specific types of care, we can curb the effects of these societal issues and even reduce the likelihood of some causal experiences.

Concepts: Cultural Competence in BH Care

Innovation Concepts

Concept 1

Provide a Support System

To establish trust, and ease individuals into trusting the system and seeking care and support them along their journey.

Concept 2

Make Culturally Competent Care Truly Accessible

To all those who currently lack it. Expand our competency as an organization.

Ramp to
Receiving Care

Support Between
Treatment

Delivering
Quality Care

Concept 1: Provide a Support System



Concept 1: Provide a Support System

The Big Idea

Support culturally diverse individuals and groups outside of traditional care interactions.

Description

In order to provide support outside of direct care delivery we must address the barriers that get in the way of individuals receiving care and benefitting from the interactions with our providers.

The first piece is an awareness and education campaign aimed at reducing stigma and helping individuals to understand the importance of seeking help and freeing them to do it. The campaign will engage providers, patients, and trusted community establishments and leaders to reduce stigma.

The second piece is a Curated Peer Group aimed at providing emotional and social support outside of care delivery. Individuals will participate in groups assembled based on shared experiences and conditions and will feature guest speakers and be guided by a group leader.

Key Audiences

- Members in African-American communities with BH conditions that are hesitant to seek care and/or feel unsupported outside of treatment.

Key Stakeholders

- Diversity and Inclusion workshop participant group
- Patient proxies

Unmet Needs It Satisfies

Lack of trust and emotional support in the community. Underlying stigma that prevents people from reaching out for help.

Key Features

- Awareness and education campaign
- Curated Peer Group

Concept 1: Provide a Support System

Business Model

Highmark and AHN funded MVP

Partner with local community organizations (with potential expansion to scale up and commercialize peer support platform to other Blues Plans)

Time considerations

- Initial pilot: 6mo–1yr
- Expand to Highmark members: 1–2 years

Success Measures

- Improvement of BH conditions
- Experience Survey surrounding care
- # of sessions attended, virtual retention data?
- Increase in demand for care

Benefits

- Increased trust and engagement with community
- Existing care becomes more effective and long lasting
- Influence expanded outside of clinical visits, more holistic reach

Innovation Type

- Underserved markets
- Customer engagement
- Channel

Concept 1: Provide a Support System

Next Steps

Pilot/Scale Considerations

- **6 months:** Establish community partnerships, develop plan for outreach and roles, learn from peer platform MVP reception
- **w/in 1yr:** Expand peer group platform to include diversity focused programming--including AHN, community champions, solidify reputation

Build/Buy/Partner Options

- Integrate virtual tools into existing platform and clinical programs with AHN
- Partner with communities/establishments/leaders to design outreach/education efforts

Potential Implementation/Ownership Teams

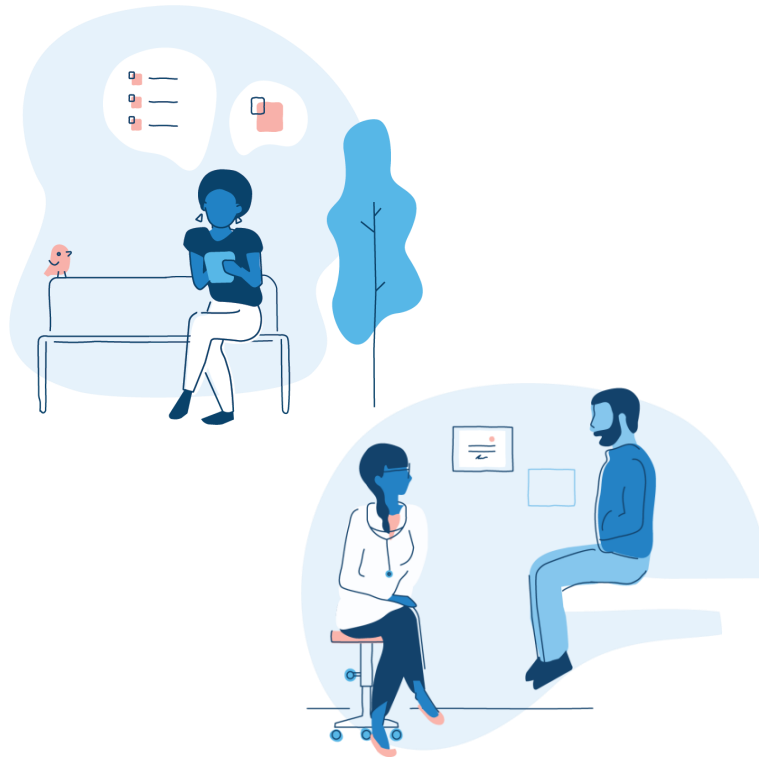
Living Health Solutions

AHN BH Clinicians and Staff

Recommended Next Actions

- Re-engagement, co creation, consistent feedback from original (and expanded) Diversity and Inclusion Workshop Participant Group (Detailed Design Phase)
- Technology build discussions about Peer Group platform
- Scoping community partnerships

Concept 2 : Culturally Competent Care Access



Provider
Education



Workforce
Diversification



Concept 2 : Culturally Competent Care Access

The Big Idea

It is necessary for providers to deliver Culturally Competent Care in order to form strong patient relationships and meet the BH needs of African-American communities and individuals.

Description

This solution consists of two major components, **Provider Education** and **Workforce Diversification**.

First is the education and training of our current provider network. Through empathy building exercises and formalized training modules, providers will learn to take the journey to become more culturally competent in their professional and personal lives.

Second is the diversification of our BH clinical workforce. This will be achieved through a modification of our recruiting/hiring practices, in order to increase interest and attract diverse providers into our network. This includes strategic partnerships with companies that put the BH needs of Black patients first, in order to better augment our provider offering outside of our footprint.

Key Audiences

- Members of African-American communities that feel unsatisfied with the care they receive because of a lack of cultural competency
 - Could manifest as a perceived lack of empathy, feeling of not being with the right provider, etc.
 - Patients receiving care but not progressing

Key Stakeholders

- Diversity and Inclusion workshop participant group
- African-American and other diverse patient proxies

Unmet Needs It Satisfies

Low efficacy of BH care for Black populations, shortage of diverse providers, low level of cultural competency in existing network,.

Key Features

- Provider Education
- Workforce Diversification

Concept 2 : Culturally Competent Care Access

Business Model

Highmark-AHN funded
Partnership(s) with companies putting the BH needs of Black people first to cocreate educational content and augment our provider network.

Time considerations

AHN footprint pilot: 6mo-1 year
Scale to Highmark members: 1 year–1.5 years

Success Measures

- BH (health status) and improvement of conditions over time
- Positive responses on patient evaluations of providers
- Proportionately diverse workforce to communities served ratio
- Increase in applications and interest from diverse providers

Benefits

- Can expand to improve care of other underserved communities and raise the level of care for everyone
- Impact our existing provider shortage

Innovation Type

Product Performance, Network

Concept 2 : Culturally Competent Care Access

Next Steps

Pilot/Scale Considerations

- **6 months:** Detailed blueprint of clinical pipeline, MVP of awareness campaign materials, target populations identified
- **w/in 1yr:** MVPs of education modules.
- **w/in 2yrs:** Expand education to all AHN BH providers, measure increase of diverse network and satisfaction + effectiveness of BH care provided to Black communities

Build/Buy/Partner Options

- Partner with companies that put the BH experience of Black people first. Co-create educational content for providers, augment our diverse provider network offering. Leverage content rather than platform.

Potential Implementation/Ownership Teams

Living Health Solutions

AHN BH Clinicians

Highmark Health BH Leads

Recommended Next Actions

Re-engagement, co creation, consistent feedback from original (and expanded) Diversity and Inclusion Workshop Participant Group (Detailed Design Phase)
Deeper research into patient experience